

Amyloid

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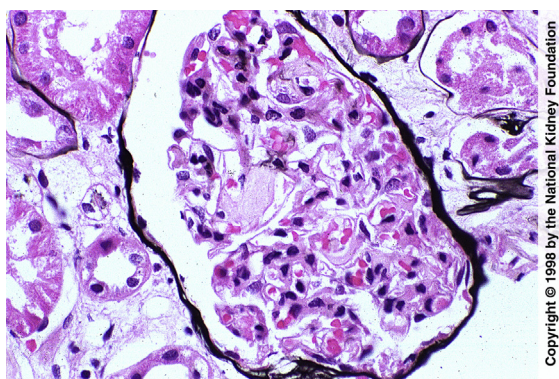


Fig 1. Acellular eosinophilic material is typical of amyloid deposits and may be found in mesangial areas or peripheral capillary loops within the glomerulus, as in this case (Jones silver stain; original magnification $\times 200$).

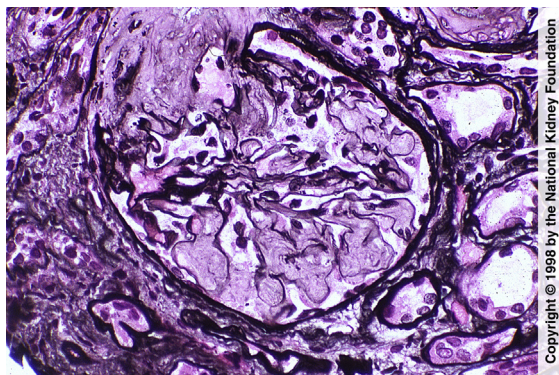


Fig 2. In a severe case of amyloidosis, massive distention of mesangial areas and distortion of peripheral capillary loops are secondary to amorphous, acellular, slightly eosinophilic pale material, which in this case also involves the arteriole at the vascular pole. This material has a more fluffy appearance than the sclerosis or mesangial matrix expansion that can occur in other conditions (Jones silver stain; original magnification $\times 200$).

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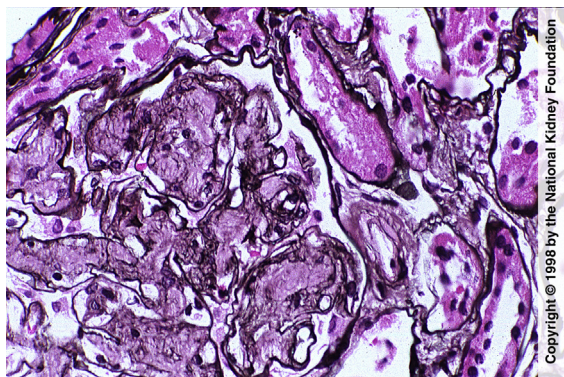


Fig 3. On high-power view of amyloid, basement membrane irregularities and feathery spikes are evident, in addition to amyloid infiltration in mesangial areas and in the arteriole to the right (Jones silver stain; original magnification $\times 400$).

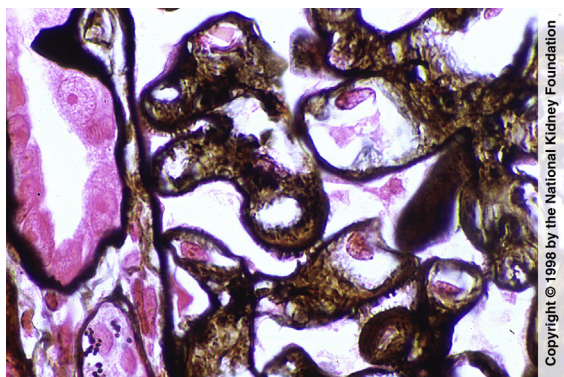


Fig 4. On high power, irregular feathery basement membrane spicules may be seen in cases of amyloid, due to the infiltration of amyloid irregularly through the basement membrane, as in this case (Jones silver stain; original magnification $\times 1,000$).

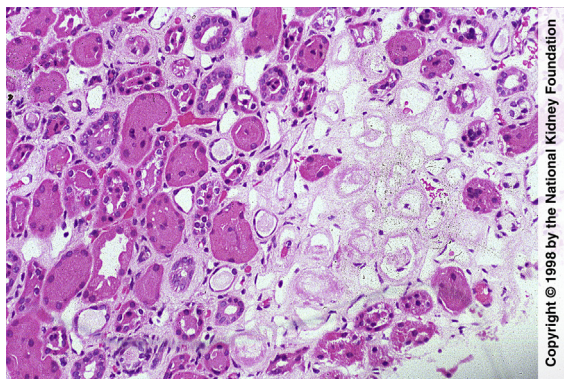


Fig 5. Amyloid may also involve the tubular interstitium, resulting in acellular pink areas that could be mistaken for areas of necrosis, as illustrated here (hematoxylin & eosin; original magnification $\times 100$).

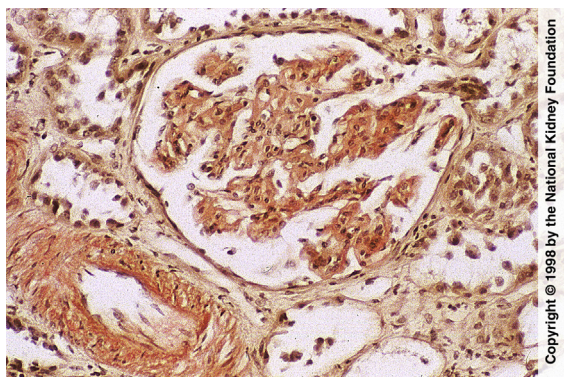


Fig 6. The gold standard for diagnosis of amyloid is Congo red positivity, shown here, which also must show apple green birefringence under polarized light (see next slide). The Congo red positivity is present in the mesangium, in the small artery, and along the basement membrane. (Congo red stain; original magnification $\times 200$).

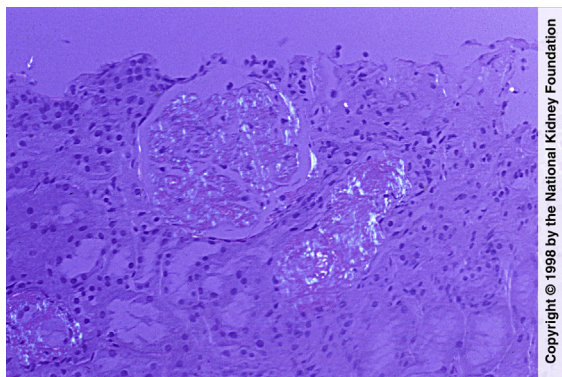


Fig 7. When the Congo red stain is viewed under polarized light, areas of amyloid show **apple green birefringence**. In this case, amyloid deposits are seen in the **mesangial areas**, the **capillary loops**, and in **vessels** (Congo red stain; original magnification $\times 100$).

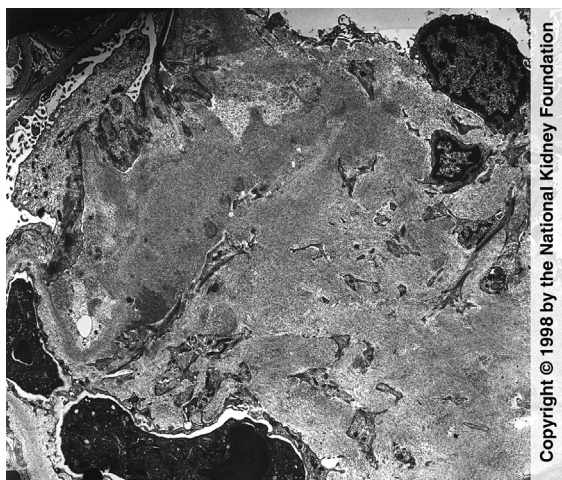


Fig 8. Electron microscopy shows **massive expansion of the mesangium** by **fibrillar material** with randomly oriented thin fibrils with a diameter of 10 to 12 nm, often **extending to basement membranes** as in this case (transmission electron microscopy; original magnification $\times 8,000$).

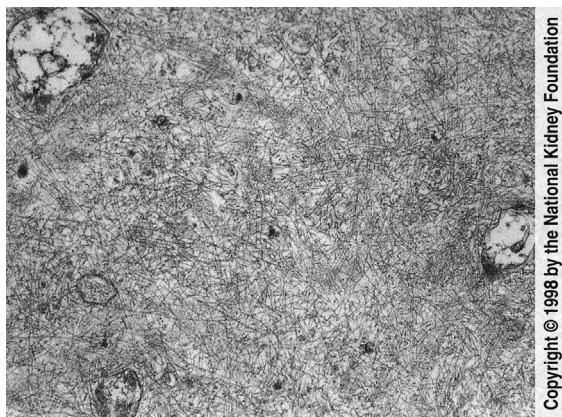


Fig 9. By electron microscopy, amyloid appears as **randomly oriented thin fibrils**, 10 to 12 nm in diameter, with a loose, flocculent background (transmission electron microscopy; original magnification $\times 51,250$).

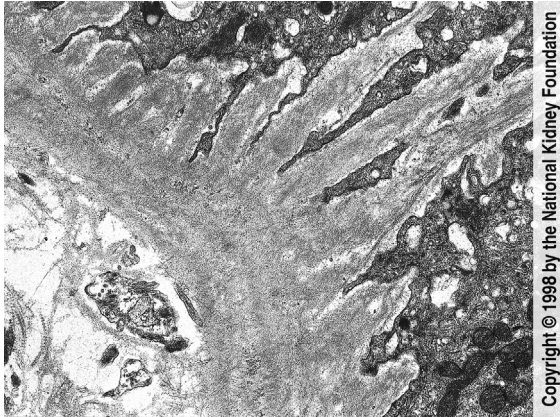


Fig 10. Amyloid infiltration through the basement membrane with resulting feathery spikes with basement membrane material and delicate amyloid fibrils are shown in this case (transmission electron microscopy; original magnification $\times 20,250$).